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FAX COVER SHEET

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TO: California Department of Managed Health Care (DMHC)
Help Center - Expedited Review Unit
FAX NUMBER: (916) 255-5241

FROM: Nicolas Hernandez

PHONE: [REDACTED]

EMAIL: [REDACTED]

DATE: August 3, 2026

TOTAL PAGES: 54

SUBJECT: EXPEDITED REVIEW REQUEST / APPEAL - ACTIVE CARE INTERRUPTION

ENROLLEE: Nicolas Hernandez

HEALTH PLAN: L.A. Care Health Plan

L.A. Care Member ID #: [REDACTED]

Medi-Cal ID # (BIC): [REDACTED]

BEHAVIORAL HEALTH PROVIDER: Carelon

DMHC CASE #: [REDACTED]44

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URGENT ATTENTION REQUIRED:

Attached is a Supplemental Appeal for Expedited Review regarding an active and unresolved access-to-care failure involving an interrupted benzodiazepine taper and documented severe withdrawal risk.

Please route directly to the Expedited Review Unit upon receipt.

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REQUEST FOR EXPEDITED DMHC REVIEW OF UNRESOLVED ACCESS-TO-CARE FAILURE

Date: August 3, 2026

To:

**California Department of Managed Health Care
Help Center**

Fax: (916) 255-5241

Email: helpline@dmhc.ca.gov

Enrollee: Nicolas Hernandez

Health Plan: L.A. Care Health Plan

L.A. CARE MEMBER ID #: [REDACTED]

MEDI-CAL ID # (BIC): [REDACTED]

BEHAVIORAL HEALTH PROVIDER: Carelon

DMHC CASE #: [REDACTED]44

RE: SUPPLEMENTAL APPEAL OF L.A. CARE'S JULY 17, 2026 RESPONSE — REQUEST FOR EXPEDITED REVIEW AND IMMEDIATE ARRANGEMENT OF CLINICALLY APPROPRIATE CARE

I request expedited DMHC review because L.A. Care's July 17, 2026 grievance response did not resolve the medically urgent access-to-care problem presented in my complaint.

My request is not merely for a general psychiatric appointment, a provider directory, or medication coverage. I am requesting actual clinical management of an established benzodiazepine taper that was interrupted after my out-of-network care ended. My treating providers documented that interruption of this treatment can cause acute withdrawal, central nervous system destabilization, and potentially life-threatening complications, including seizures.

Although L.A. Care supplied the names of practices with purported appointment availability, no clinician accepted responsibility for the interrupted taper, reviewed and adopted the existing treatment plan, completed a clinical handoff, or implemented an interim treatment plan.

A future referral does not constitute present access to medically necessary care. Benzodiazepine withdrawal does not pause while a referral is processed.

1. THE REQUESTED SERVICE AND THE END OF [REDACTED] CARE

Before treatment was interrupted, [REDACTED] had safely reduced my diazepam dosage from 20 mg per day to 12.5 mg per day. The May 28, 2026 visit summary documented that I was stable on the current regimen, had no interdose-withdrawal symptoms, and was continuing to stabilize after the prior reduction.

When financial barriers prevented me from continuing to pay for out-of-network treatment, [REDACTED] did not immediately terminate care. The practice continued providing clinical guidance and taper management for approximately 30 additional days without payment while I attempted to obtain appropriate in-network care.

At the conclusion of that unpaid grace period, active clinical management ended on June 8, 2026. [REDACTED] then issued a final 28-day diazepam prescription rather than its usual seven-day prescription because locating an appropriate in-network provider was expected to be difficult. Carepoint Pharmacy's dispensing history confirms that this final prescription was filled on June 10, 2026 as a 28-day supply.

[REDACTED] therefore created two separate transition safeguards: approximately one month of unpaid continued clinical care, followed by a final 28-day medication buffer. The final prescription temporarily extended medication availability until approximately July 8, 2026, but it did not extend the treatment relationship or provide continued monitoring, dose-management guidance, or clinical supervision after June 8.

Despite the additional transition time [REDACTED] created, L.A. Care and Carelon did not establish a receiving clinician, complete a clinical handoff, or implement an interim treatment plan before active care ended and the final medication supply was exhausted.

2. L.A. CARE'S JULY 17 RESPONSE DID NOT ESTABLISH CARE

L.A. Care's July 17 response identified the following practices:

- Brain Health USA
- Inland Psych Medical Group
- Healing Journeys Mental Health

The response listed appointment availability but did not identify a named clinician who had:

- received and reviewed my treatment records;
- agreed to assume the existing taper;
- determined that they were clinically capable of managing it;
- provided an interim treatment plan; or
- completed a warm handoff from the prior treating practice.

L.A. Care's conclusion therefore addressed theoretical appointment availability, not actual availability of the specific medically necessary service requested.

L.A. Care also focused on whether provider nonpayment caused the interruption. That dispute does not resolve the present access problem. Whatever the precise financial mechanism, the objective facts are that active taper management ended, the temporary final medication supply was exhausted, and no receiving clinician assumed responsibility for treatment.

3. TWO OF THE LISTED PRACTICES HAD ALREADY BEEN ATTEMPTED & CARELON PREVIOUSLY RECYCLED THEM

Brain Health USA

Brain Health USA was not a new or untested option. I previously sought medication-management and benzodiazepine-taper care through that organization. Attached contemporaneous records and licensing-board complaint submissions document encounters in which Brain Health USA providers either declined to manage the requested taper or failed to provide continuity during medication-management problems. Referring me back to the same organization without identifying a clinician who had affirmatively reviewed and accepted the case did not resolve the network-access problem.

Inland Psych Medical Group

Inland Psych was also previously attempted. During the May 26, 2026 encounter with Kenneth Ezenwa, NP, no transfer of the taper was completed. Records reflect that the provider did not begin the appointment with access to my relevant medication history, and the encounter did not result in the provider assuming management of the diazepam taper.

Healing Journeys Mental Health

L.A. Care identified an appointment date at Healing Journeys but did not establish that any named clinician had reviewed the case, agreed to manage the established taper, or proposed an interim treatment plan.

VIOLATION OF CALIFORNIA TIMELY ACCESS TO CARE STANDARDS (28 CCR § 1300.67.2.2)

Furthermore, the appointment dates provided in L.A. Care's July 17 response violate state timely access standards. Because this matter was handled as an expedited grievance involving an interrupted, high-risk medication taper, California law mandates that urgent care appointments be provided within 48 to 96 hours (28 CCR § 1300.67.2.2).

L.A. Care's offer of an appointment at Healing Journeys on July 28—14 calendar days after my July 14 grievance—and Inland Psych "within 15 business days" blatantly fails California's urgent access requirements. Offering a two-to-three-week delay to a patient actively experiencing unmanaged benzodiazepine withdrawal is legally non-compliant and clinically unsafe.

Carelon's Prior Knowledge & Network Recycling

On June 23, 2026—weeks prior to L.A. Care's July 17 grievance response and well before my final medication supply was exhausted—I called Carelon Behavioral Health directly (1-877-344-2858, 22-minute call documented in Exhibit 13). During that call, Carelon searched its provider system and could only offer Brain Health USA and Inland Psych. Carelon was aware as early as June 23 that these were the only options in their system, despite my prior failed attempts to secure taper management through them. L.A. Care's July 17 response merely recycled these exact same dead-end options, demonstrating that neither L.A. Care nor Carelon took meaningful action to locate a willing and qualified clinician.

4. THE JULY 29 BARTZ-ALTADONNA EVALUATION CONFIRMED THAT THE CARE GAP REMAINED UNRESOLVED

On July 29, 2026, more than seven weeks after active taper management ended and approximately three weeks after the final medication supply was exhausted, I was evaluated at Bartz-Altadonna Community Health Center.

The progress note acknowledged:

- that my last diazepam dose was July 8;
- multiple recent emergency-department visits involving benzodiazepine withdrawal;
- worsening anxiety after the medication ended; and
- a history of repeated withdrawal.

The provider declined to prescribe benzodiazepines and entered a referral to addiction medicine. The resulting assessment and plan consisted primarily of routine laboratory testing, the referral, and a later telephone follow-up regarding laboratory results.

No clinician assumed management of the interrupted taper. No confirmed receiving appointment was documented. No completed handoff occurred. No bridge, alternative medication strategy, or other interim clinical-management plan addressing the acknowledged withdrawal problem was established.

A future referral does not constitute present care. Benzodiazepine withdrawal does not wait for a referral.

5. DOCUMENTED CLINICAL RISK

My treating psychiatric practice issued a written Medical Necessity Statement for Expedited Review on April 20, 2026.

The statement explains that interruption of care, delays in access to prescribed medications, or discontinuation caused by financial barriers carry clinically significant risks, including:

- acute withdrawal;
- destabilization of the central nervous system; and

- potentially life-threatening complications such as seizures.

It further states that a carefully managed, gradual taper and timely, uninterrupted access to care are medically necessary. After my established treatment ended, I required multiple emergency-department evaluations involving benzodiazepine withdrawal. Emergency departments did not replace the longitudinal clinician needed to supervise and manage the taper.

6. PROCEDURAL VIOLATIONS AND DMHC ADMINISTRATIVE OMISSION

A. L.A. Care's Delayed Handling of the Expedited Grievance

L.A. Care acknowledged receiving my expedited grievance on July 14, 2026 and expressly stated that I would receive a resolution within 72 hours[cite: 2]. The resolution letter is dated July 17, 2026, but it did not reach me until July 23, 2026, as documented by USPS Informed Delivery. Because the grievance concerned an active interruption of medically necessary treatment, delayed communication deprived the expedited process of practical value.

I request that DMHC verify the transmission, production, and mailing dates to determine whether the required written disposition was actually issued and provided within the mandatory expedited timeframe.

B. DMHC Procedural Error: Improper Merger & Omission of Urgent IMR Request

On June 19, 2026, I called the DMHC Help Center (1-888-466-2219, 14-minute call documented in Exhibit 14). During this call, DMHC staff confirmed that my two separate online portal submissions—an Urgent IMR Request regarding an interrupted psychiatric taper/network failure (Exhibit 15) and a Consumer Complaint regarding transportation timelines (Exhibit 4)—were being merged under DMHC Case # [REDACTED] 44.

However, DMHC's June 26, 2026 response letter (Exhibit 11) completely omitted the Urgent IMR filing. The Department addressed *only* the transportation complaint, administratively erasing my high-risk clinical network failure complaint. I formally request that DMHC correct this administrative oversight and immediately process the omitted Urgent IMR / Network Access appeal.

7. REQUESTED DMHC ACTION

I respectfully request that DMHC:

1. Accept this matter for expedited handling because the interruption of an established benzodiazepine taper presents a documented risk of serious withdrawal complications.
2. Determine that a provider directory, appointment list, or future referral does not, by itself, establish access to the specific medically necessary service requested.
3. Require L.A. Care and Carelon to identify a specific qualified clinician who:
 - has received and reviewed my relevant records;

- has affirmatively agreed to evaluate and assume responsibility for the taper;
- can provide care within a clinically appropriate timeframe; and
- will implement an interim plan rather than leave the condition unmanaged during referral processing.

4. If no appropriate and willing in-network clinician is immediately available, require L.A. Care to arrange and authorize out-of-network treatment with my established treating practice, or another qualified provider, through a Single Case Agreement or any other contracting and payment mechanism necessary to prevent further interruption.

5. Require L.A. Care to arrange an immediate clinical safety and transition plan while permanent placement is being completed.

6. Require a designated case manager and written coordination plan identifying the receiving provider, appointment date, record transfer status, and interim arrangements.

7. Review my request for reimbursement of documented out-of-pocket treatment expenses resulting from the absence of timely in-network care.

8. Investigate L.A. Care's handling of the July 14 expedited grievance and determine whether the required disposition was timely issued.

CONCLUSION

No clinician assumed responsibility when active taper management ended June 8.

██████████ created a temporary bridge, but L.A. Care failed to establish care on the other side of it. A final medication supply was not continued clinical care, and a future referral did not replace the treatment that had already ended. Benzodiazepine withdrawal does not wait for a referral.

I request immediate DMHC intervention to require an actual, completed arrangement for clinically appropriate care.

Respectfully submitted,

Nicolas Hernandez

████████████████████

██████████ CA 9 ██████████

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EXHIBIT INDEX

Exhibit 1: L.A. Care acknowledgment of expedited grievance dated July 15, 2026, stating that a resolution would be received within 72 hours (Page 1)

Exhibit 2: L.A. Care grievance resolution dated July 17, 2026 (Pages 2-4)

Exhibit 3: USPS Informed Delivery record documenting delivery of the L.A. Care mailpiece on July 23, 2026 (Pages 5-7)

Exhibit 4: [REDACTED] Medical Necessity Statement for Expedited Review dated April 20, 2026 and visit summaries and communications documenting the taper. (Pages 8-10)

Exhibit 5: Screenshots of [REDACTED] termination of active clinical management on June 8, 2026, and issuance of the final prescription (Pages 11-12)

Exhibit 6: Carepoint Pharmacy dispensing history confirming that the final diazepam prescription was filled June 10, 2026 as a 28-day supply (Page 13)

Exhibit 7: Medical Board of California Complaint Brain Health USA, Dr. Virk (#8002026127107) and Dr. Mahmoud (#8002026126991) (Pages 14-21)

Exhibit 8: California Board of Registered Nursing Complaint Inland Psych, Kenneth Ezenwa (#4002026009197) (pages 22-25)

Exhibit 9: Bartz-Altadonna progress note dated July 29, 2026 (Pages 26-28)

Exhibit 10: Emergency-department records from July 2026 (Pages 29-31)

Exhibit 11: DMHC Response Letter dated June 26, 2026, showing the Department addressed *only* non-emergency transportation and completely omitted the Urgent IMR request (Pages 32-33)

Exhibit 12: Call log record—June 23, 2026, documenting a 22-minute call to Carelton Behavioral Health (1-877-344-2858) prior to medication exhaustion, during which Carelton identified only Brain Health USA and Inland Psych—confirming that L.A. Care's July 17 response merely recycled previously attempted dead-end options. (Page 34)

Exhibit 13: Call log record—June 19, 2026, documenting a 14-minute call to the DMHC Help Center (1-888-466-2219) regarding the merger of portal submissions. (Page 35)

Exhibit 14: Text of Original Urgent IMR Portal Submission detailing the life-threatening taper interruption and request for directed physician referral. (Page 36)

Exhibit 15: Bartz-Altadonna Progress Note—October 3, 2025 (FNP Jasmin Garcia), establishing long-standing internal clinical documentation of severe seizure risk upon benzodiazepine discontinuation ("Reports seizing without any clonazepam"). (Pages 37-39)

Exhibit 16: California Board of Registered Nursing Complaint (#4002026005943) filed February 19, 2026, documenting Bartz-Altadonna's failure to manage documented seizure risk and instructions to utilize emergency facilities for maintenance. (Pages 40-44)

Exhibit 17: L.A. Care Grievance Resolution Letter dated November 4, 2025 (Date of Service: October 05, 2025), establishing that L.A. Care had written notice of network provider shortages nearly 10 months prior and officially sanctioned the use of emergency rooms for ongoing prescription maintenance. (Page 45)

Exhibit 1

July 15, 2026

NICOLAS HERNANDEZ

CA 9

Member Name: NICOLAS HERNANDEZ

Member ID:

Reference Number: 03

Line of Business: MCLA

Date of Service: July 14, 2026



Dear NICOLAS HERNANDEZ,

Thank you for bringing your concern(s) to our attention on July 14, 2026. L.A. Care will review your expedited grievance and provide a response to you. You will receive a resolution on the matter within 72 hours from the time we received your request.

If your matter is urgent, such as those involving a serious threat to your health, you may call DMHC right away without first filing a grievance or appeal with L.A. Care. Please see the "Department of Managed Health Care" section on how to file a grievance with DMHC.

The California Department of Managed Health Care is responsible for regulating health care service plans. If you have a grievance against your health plan, you should first telephone your health plan at 1-888-839-9909 and use your health plan's grievance process before contacting the department. Utilizing this grievance procedure does not prohibit any potential legal rights or remedies that may be available to you. If you need help with a grievance involving an emergency, a grievance that has not been satisfactorily resolved by your health plan, or a grievance that has remained unresolved for more than 30 days, you may call the department for assistance. You may also be eligible for an Independent Medical Review (IMR). If you are eligible for IMR, the IMR process will provide an impartial review of medical decisions made by a health plan related to the medical necessity of a proposed service or treatment, coverage decisions for treatments that are experimental or investigational in nature and payment disputes for emergency or urgent medical services. The department also has a toll-free telephone number (1-888-466-2219) and a TDD line (1-877-688-9891) for the hearing and speech impaired. The department's internet website <http://www.dmhc.ca.gov> has complaint forms, IMR application forms and instructions online.

AG0128 0424

Exhibit 2



L.A. Care
HEALTH PLAN

For All of L.A.

July 17, 2026

NICOLAS HERNANDEZ

██████████
██████████ CA, 9 ██████████

Member Name: NICOLAS ████████ HERNANDEZ

Member ID: ██████████

Reference Number: ████████03

Line of Business: Medi-Cal

Date of Service: July 14, 2026

Regarding: Quality of care



Dear NICOLAS HERNANDEZ,

Thank you for bringing your concern(s) to our attention. L.A. Care Health Plan received your grievance on July 14, 2026. The following describes the results of our review:

Concern/s:

Your grievance stated that you were experiencing difficulties accessing behavioral health services and maintaining continuity of care during a medically supervised benzodiazepine taper. You reported that you were without a prescribing provider, had a limited supply of medication remaining, and believed that your treatment was interrupted because your previous behavioral health provider was not being paid.

Resolution:

We completed a thorough review of your concerns, including information provided by you and information obtained from Carelon Behavioral Health, our behavioral health administrator.

- Carelon Behavioral Health did not identify call records or claims related to your concern dated July 14, 2026.
- On July 15, 2026, Carelon submitted an urgent request for its Clinical Team to conduct a risk assessment.
- Carelon's Clinical Team attempted to contact you by telephone on three separate occasions at the phone number on file ((661) 648-7301). They were unable to leave voicemail messages because your voicemail mailbox was full.
- Carelon also identified in-network behavioral health prescribing providers with appointment availability, including:
 - o Brain Health (Telchealth) – Earliest available appointments on July 21, 2026

- o Inland Psych Medical Group, Inc. – In-person appointments available within 15 business days
- o Healing Journeys Mental Health – Earliest available appointment on July 28, 2026

We also attempted to contact you on July 16, 2026 (11:27 a.m. and 12:14 p.m.) and July 17, 2026 (10:46 a.m.). Our calls were not answered, and we were unable to leave a voicemail because your mailbox was full.

Based on the information available during our review, we could not substantiate that L.A. Care or Caredon Behavioral Health caused an interruption in your care due to nonpayment of your previous provider. Our investigation did not identify evidence supporting that allegation.

However, we recognize the importance of uninterrupted behavioral health treatment, particularly during medication management. Caredon identified available in-network providers who are accepting appointments, and outreach was initiated to assist you with connecting to care and completing a clinical risk assessment.

If you still need assistance obtaining an appointment with a behavioral health prescribing provider or continue to have concerns about your medication or treatment, please contact L.A. Care Member Services as soon as possible so we can assist you.

After careful review, your case has also been forwarded to our Quality Improvement Department to determine whether appropriate medical standards of care were provided. The Quality Improvement Department will investigate and review all relevant information and will take any necessary corrective actions to support ongoing quality improvement.

Please note that, due to federal and state confidentiality laws—including California Evidence Code Section 1157, Health and Safety Code Section 1370, and California Business and Professions Code Section 805—as well as medical peer review requirements, we are unable to share details of the investigation or the outcome of any peer review activities.

We want to assure you that all quality-of-care concerns are carefully tracked, monitored, and addressed as part of our comprehensive Quality Improvement Program.

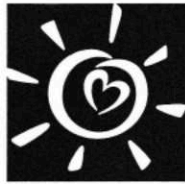
Thank you for taking the time to share your experience. Your feedback helps us improve the care and services we provide to our members.

We hope that you are satisfied with this review. If you are not satisfied with the results, you may file an appeal to the Department of Managed Health Care.

California Department of Health Care Services (DHCS) Office of the Ombudsman

You may also call the Ombudsman Office of the California Department of Health Care Services (DHCS) for help. The Ombudsman Office helps Medi-Cal beneficiaries to fully use their rights and responsibilities as a member of a managed care plan. To find out more, call toll-free 1-888-452-8609.

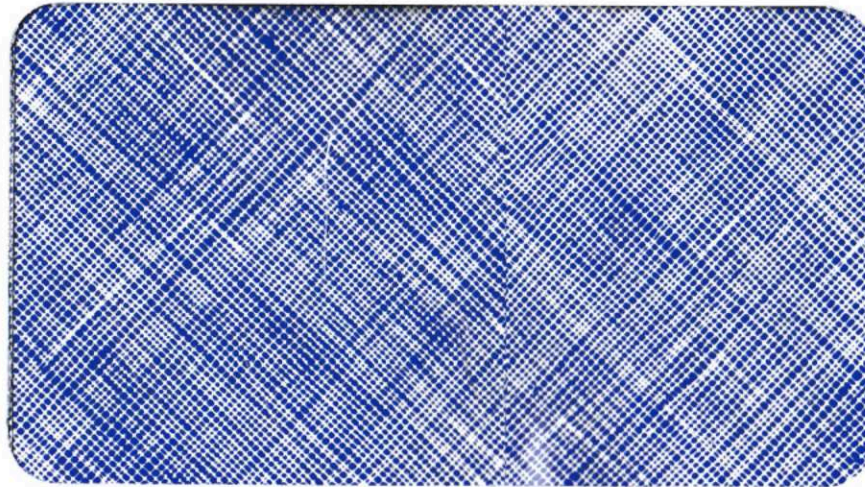
AGXXXX 0620



L.A. Care
HEALTH PLAN®

lacare.org

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LA0366SWEAG 09/19-PO

9 3 2008



Exhibit 3



N H <[REDACTED]@gmail.com>

Your Daily Digest for Thu, 7/23 is ready to view

1 message

USPS Informed Delivery <USPSInformedelivery@email.informedelivery.usps.com>

To: [REDACTED]@gmail.com

Thu, Jul 23, 2026 at 7:27 AM



COMING TO YOU SOON

Hi, Nicolas!

You have 1 mailpiece(s) and 1 inbound package(s) arriving soon.

Thursday

23 July 2026

1

Mailpiece(s)

1

Package(s)

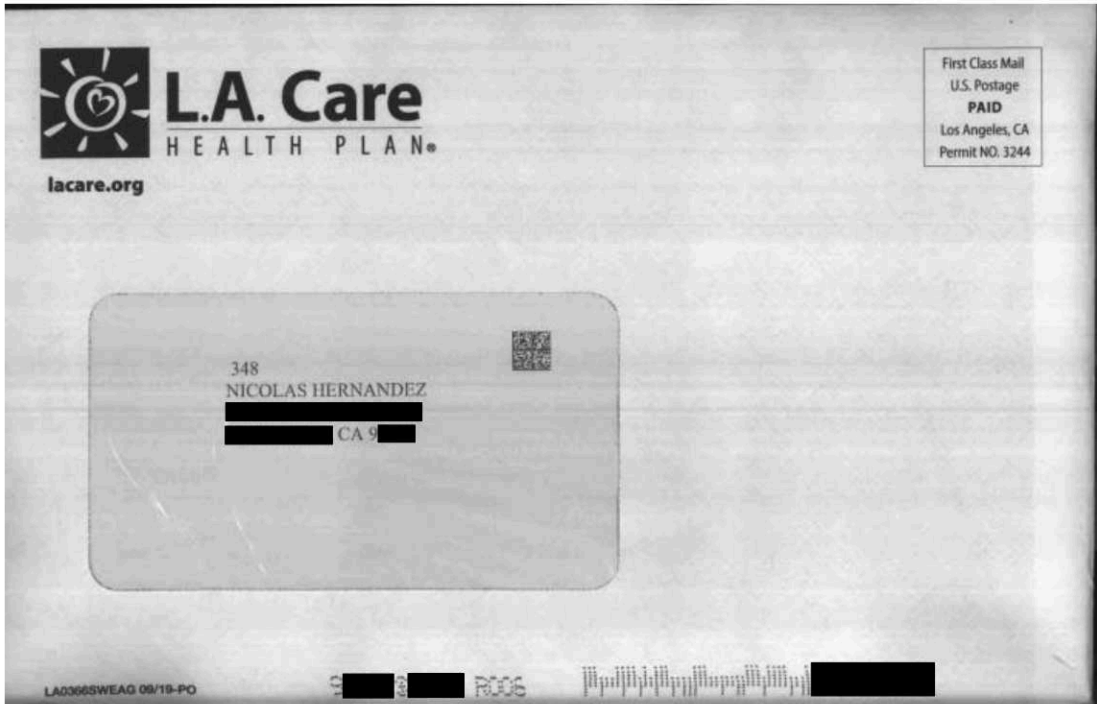


MAIL

[View Dashboard](#)

Expected Today

1 item(s)



PACKAGES

[View Dashboard](#)

Expected Today

1 item(s)

FROM:

Expected 1-2 Days

0 item(s)

No packages are available to display.

Awaiting From Sender

0 item(s)

No packages are available to display.

Outbound

0 item(s)

No packages are available to display.

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Exhibit 4

[REDACTED] MD

[REDACTED]
[REDACTED] Street, # [REDACTED]
[REDACTED] CA 9 [REDACTED][REDACTED]
[REDACTED]**FROM THE OFFICE OF [REDACTED] MD MPH****April 20, 2026****Patient:** Nicolas Hernandez**Date of Birth:** [REDACTED]**Re:** Medical Necessity Statement for Expedited Review

I am writing on behalf of our patient, who is currently under care for a medically supervised benzodiazepine taper, which is being conducted due to physiological dependence and a history that includes significant withdrawal symptoms and elevated risk during medication transitions.

Interruptions in care, including delays in access to prescribed medications or discontinuation of treatment due to financial barriers, carry a clinically significant risk. These risks include acute withdrawal symptoms, destabilization of the central nervous system, and the potential for life-threatening complications such as seizures. Additionally, abrupt disruption may negatively impact mental health stability and the patient's ability to maintain daily functioning.

Given the nature of benzodiazepine dependence and the need for a carefully managed, gradual taper, timely and uninterrupted access to care is medically necessary. Delays associated with standard processing timelines may pose a risk to the patient's physical and mental health, as well as their ability to maintain and regain functional stability.

Based on the above, it is my clinical opinion that this situation meets criteria for expedited review. Please feel free to contact me if additional information is needed.

Sincerely,

[REDACTED]

[REDACTED] PMHNP-BC

Collaborating Physician: [REDACTED] MD MPH, Family Medicine

[REDACTED] MD

[REDACTED]
[REDACTED] Street, # [REDACTED]
[REDACTED] CA 9 [REDACTED]

[REDACTED]
[REDACTED]

FROM THE OFFICE OF [REDACTED] MD MPH

Visit Summary: March 27, 2026

Overall

- Ongoing stress related to insurance reimbursement and legal action against LA Care; DHCS Conlan Department involved
- Filed small claims due to lack of response for expedited hearing
- Financial constraints impacting ability to continue [REDACTED] program if reimbursement not approved
- Remains highly engaged in care and motivated to continue taper if financially able

Medical

- No withdrawal symptoms or instability
- No new medical concerns discussed

Taper

- Previous dose, diazepam 20 mg/day, 01/18/2026
- Current dose, diazepam 17.5 mg/day, started 03/03/2026
- Planned next dose, diazepam 15mg/day, planned start 04/06/2026
- Anticipating a dose hold of approximately 4 weeks prior to next reduction to diazepam 10mg/day, anticipated on or after 04/17/2026

Refill

- Prescription sent to CVS 03/30/2026 for diazepam 17.5 mg/day, 7 days
- Subsequent Rx send to CVS for diazepam 15 mg/day, 7 days

Follow-Up Video Visit: Friday, May 1, 12:30p (PT)

[REDACTED] MD

[REDACTED]
[REDACTED] Street, # [REDACTED]
[REDACTED] CA 9 [REDACTED]

[REDACTED]
[REDACTED]

FROM THE OFFICE OF [REDACTED] MD MPH

Visit Summary: Feb 25, 2026

Overall

- More stable compared to prior week
- Pharmacy filled last prescription two days early creating buffer
- No current pharmacy conflicts
- DHCS reimbursement form 4521 received and under review
- If reimbursement approved plans to extend taper to approximately 12 months
- Feels ready to move forward with next reduction
- Prefers smaller reductions to avoid rebound or destabilization

Medical

- No acute withdrawal symptoms reported
- Continues healing from prior pharmacy-related stress episode
- No instability noted at current dose

Taper

- Current dose, diazepam 20 mg/day, date started 01/18/2026
- Planned next dose, diazepam 17.5 mg/day, planned start date 03/03/2026

Refill

- Prescription sent to CVS 03/02/26
- Update 03/09/26: Subsequent Rx sent to CVS for diazepam 17.5 mg/day, 7 days

- Follow-Up Video Visit: Friday, March 27, 11:30a (PT)

Messages

**Exhibit 5**

Today 3:03 PM

Visit Summary: May 28, 2026

Overall

- Ongoing legal and reimbursement efforts related to coverage denial and treatment costs
- Preparing to refile small claims case if current tort claim is not resolved within response period
- Attempted transfer to in-network provider was unsuccessful due to admin errors
- Filed OCR complaint after intake paperwork was sent to father despite no authorization or emergency contact designation
- Difficulty obtaining transportation assistance for pharmacy visits continues

Medical

- Stable on current medication regimen
- No current interdose withdrawal symptoms
- Transition from previous dose was somewhat difficult initially but continuing to stabilize



Type a message



Chat



Dashboard



Prescriptions



Appointments



Settings



Messages



intake paperwork was sent to father despite no authorization or emergency contact designation

- Difficulty obtaining transportation assistance for pharmacy visits continues

Medical

- Stable on current medication regimen
- No current interdose withdrawal symptoms
- Transition from previous dose was somewhat difficult initially but continuing to stabilize

Taper

- Previous dose, diazepam 17.5 mg/day, 03/03/2026
- Previous dose, diazepam 15mg/day, 04/06/2026
- Current dose, diazepam 12.5mg/day, started 05/11/2026

Refill

- Next rx send, 06/08/206 - sending month supply - last rx

- Follow-Up Video Visit: No appt at this time; subscription will be cancelled

Type a message



Chat



Dashboard



Prescriptions



Appointments



Settings



Exhibit 6**Carepoint Pharmacy - Palmdale, CA**

2270 E Palmdale Blvd, Ste K

/L

Palmdale, CA 93550-1339

(661) 480-2373

Patient: Hernandez, Nicolas

[REDACTED]

[REDACTED] CA 9 [REDACTED]

DOB:

[REDACTED]

[REDACTED]

01/01/2026 to 06/11/2026

Drug	NDC	Gnrc	Rx	Rf	Rph	Date	Qty	Patient Paid	Third Party Plan	Daw	Day	Prescriber
Diazepam 5 Mg Tablet	00172-3926-60	Y	[REDACTED] 373	0	CO	06/10/2026	72.00000	0.00	14.48 CAWM	0	28	[REDACTED]
Diazepam 5 Mg Tablet	00172-3926-60	Y	[REDACTED] 963	0	CO	06/01/2026	18.00000	0.00	13.52 CAWM	0	7	[REDACTED]
Diazepam 5 Mg Tablet	00172-3926-60	Y	[REDACTED] 641	0	MF	05/26/2026	18.00000	0.00	13.52 CAWM	0	7	[REDACTED]
Sertraline Hcl 50 Mg Tablet	65862-0012-05	Y	[REDACTED] 648	0	CO	05/26/2026	30.00000	0.00	13.84 CAWM	0	30	Ezenwa, Kenneth
Diazepam 5 Mg Tablet	00172-3926-60	Y	[REDACTED] 294	0	CO	05/19/2026	18.00000	0.00	13.52 CAWM	0	7	[REDACTED]
Diazepam 5 Mg Tablet	00172-3926-60	Y	[REDACTED] 018	0	CO	05/13/2026	18.00000	0.00	13.52 CAWM	0	7	[REDACTED]
Diazepam 5 Mg Tablet	00172-3926-60	Y	[REDACTED] 454	0	CO	05/07/2026	21.00000	0.00	13.57 CAWM	0	7	[REDACTED]
Diazepam 5 Mg Tablet	00172-3926-60	Y	[REDACTED] 055	0	MF	04/27/2026	21.00000	0.00	13.57 CAWM	0	7	[REDACTED]

Total Patient Paid: \$0.00**No. Of Rxs:** 8**Signature**

NCPDP: 5681691

Tax ID: 861875173

Board:	Medical Board of California
License Type:	Physician's and Surgeon's
Complaint Number:	8002026127107
Incident Date:	11/11/2025

Description:

I scheduled an intake via telehealth with Dr. Inderpreet Virk, operating as the lead psychiatrist for Brain Health USA. I sought care based on the clinic's explicit advertisements stating they specialize in complex substance issues and benzodiazepine tapering. I presented seeking a safe, medically supervised taper off of street-sourced Xanax (historically up to 22mg, at the time of meeting, 6mg).

Despite being a Board-Certified Addiction Psychiatrist, Dr. Virk exhibited a severe departure from the standard of care:

Refusal to Treat / Patient Abandonment: Dr. Virk explicitly refused to initiate a standard medical tapering protocol, stating my case was "too much of a liability" and a "sticky situation" because my prior usage was street-sourced and lacked pharmaceutical documentation. Refusing to medically manage a patient actively seeking to transition from illicit benzodiazepines to safe medical oversight—while knowing the fatal risks of abrupt benzodiazepine withdrawal—constitutes gross negligence and a fundamental failure to perform the duties of an addiction specialist.

Misleading Advertising (Bait-and-Switch): Brain Health USA specifically advertises specialized benzodiazepine tapering to Medi-Cal/L.A. Care patients. However, their lead addiction psychiatrist outright refuses to treat patients with a history of street procurement, which encompasses a massive demographic of the addiction population they claim to serve.

Substandard Psychiatric Assessment: After denying primary care for the benzodiazepine dependency, Dr. Virk reluctantly prescribed a 10-day Suboxone bridge at my desperate request. He then concluded the session by rapidly asking if I felt suicidal or felt like hurting or others, utilizing a dismissive, "box-checking" approach to a severe psychiatric safety assessment for a highly vulnerable patient he had just denied comprehensive care to.

I request the Board investigate Dr. Virk's refusal to provide standard addiction medicine care, his patient abandonment due to "liability," and the misleading nature of the clinic's advertised specialties.

Business Name/Facility Name:	Brain Health USA (Telehealth Only)
Address Line 1:	1225 W 190th St Ste 280
City:	Gardena
State:	California
Zip:	90248
Hospital:	No
Home:	No
Other:	No
If you responded 'Yes' to the previous question, please provide a description of the location of the incident.	Telehealth

Have you filed a complaint with any other government agency regarding this incident?	No
Does this complaint concern a child custody issue?	No
Was the person named in this complaint appointed by the court to prepare a custody recommendation to the court?	No
Have you tried to resolve or mediate this complaint?	No
Reason for treatment:	Benzodiazepine (Xanax) Taper
Patient Name:	Nicolas Hernandez
Date of Birth:	██████████ (mm/dd/yyyy)
Relationship to the patient:	Self
Substandard Care (e.g. misdiagnosis, negligent treatment, delay in treatment, etc.)	Yes
Prescribing Issues (e.g. excessive/under prescribing, Internet)	Yes
Unlicensed Provider or Aiding/Abetting unlicensed practice	No
Sexual Misconduct	No
Provider (e.g. Physician, Psychologist, Psychotherapist etc.) Impairment (e.g. drug, alcohol, mental physical)	No
Unprofessional Conduct (e.g. breach of confidence, record alteration, fraud, misleading advertising, arrest or conviction)	Yes

Office Practice (e.g. failure to provide patient/medical records to patient, failure to sign death certificate, patient abandonment):

Yes

If the complaint is medical/psychological/psychotherapy treatment related, please indicate the patient has been examined/treated by another provider/professional for this same condition?:

Yes

If you responded 'Yes' to the previous question, please include provider name, address, and treatment dates.

[REDACTED]

Have you initiated or filed legal action regarding this incident?

No

What is the desired outcome of your complaint submission?

Investigate refusal of care & negligence

License Type:

Physician's and Surgeon's

First Name:

INDERPREET

Second Name:

SINGH

Last Name:

VIRK

Address:

**7892 Kelly Canyon PI
DUBLIN, CA
94568-1395
US**

First Name:

Nicolas

Last Name:

Hernandez

Gender:

Male

Address:

[REDACTED]

LOS ANGELES

[REDACTED] **CA**

9 [REDACTED]

US

Phone Number:

[REDACTED]

E-mail Address:

[REDACTED]

Online Complaint Summary

2/14/26 12:17 PM

Page 1 of 4

Board: Medical Board of California
License Type: Physician's and Surgeon's
Complaint Number: 8002026126991
Incident Date: 11/23/2025
Description: I was a patient of Dr. Mahmoud Aborabeh (License A 171284) at Brain Health USA for psychiatric medication management (Suboxone & Gabapentin).

1. Patient Abandonment Causing Medical Emergency:

During a critical pharmacy hold on my Suboxone prescription (Nov 23 – Dec 9), Dr. Aborabeh refused to speak with the pharmacist to release the medication, stating "I've done my part". His refusal to act forced me into withdrawal, requiring me to seek emergency care at Palmdale Regional Medical Center on 12/06/2025 (MRN: PRM[REDACTED]27) just to obtain a 3-day bridge prescription to maintain stability.

2. Unprofessional Conduct (Telehealth):

Dr. Aborabeh consistently failed to maintain professional standards. On my first telehealth appointment with him he appeared on the video call wearing pajamas and sitting in bed, demonstrating a total lack of respect for the medical setting.

3. 'Pill Mill' Operation:

The practice appears to prioritize high-volume prescribing over safety. I was informed Dr. Aborabeh had reached his federal patient cap for Suboxone, which explains his refusal to provide follow-up appointments or manage the pharmacy crisis.

4. Financial Harm:

Because Dr. Aborabeh refused to assist with the pharmacy or offer an urgent follow-up (citing a 2-week wait), I was forced to go to the ER for a 72 hour Suboxone bridge and then seek a new private provider out of pocket.

Incident Address 1

Business Name/Facility Name:

Brain Health USA (Telehealth Only)

Address Line 1: **14541 Delano St**
City: **Van Nuys**
State: **California**
Zip: **91411**
Hospital: **No**
Home: **No**
Other: **Yes**
If you responded 'Yes' to the previous question, please provide a description of the location of the incident. **Telehealth**

Additional Complaint Information 1

Have you filed a complaint with any other government agency regarding this incident? **No**

Does this complaint concern a child custody issue? **No**

Was the person named in this complaint appointed by the court to prepare a custody recommendation to the court? **No**

If children are involved in this case, do you have joint legal custody of the child/children involved in this case? **No**

Have you tried to resolve or mediate this complaint? **No**

Reason for treatment: **Medication Management**

Patient Name: **Nicolas Hernandez**

Date of Birth: **(mm/dd/yyyy)**

Relationship to the patient: **Self**

Substandard Care (e.g. misdiagnosis, negligent treatment, delay in treatment, etc.) **Yes**

Prescribing Issues (e.g. excessive/under prescribing, Internet) **Yes**

Unlicensed Provider or Aiding/Abetting unlicensed practice **No**

Sexual Misconduct **No**

Provider (e.g. Physician, Psychologist, Psychotherapist etc.) Impairment (e.g.drug, alcohol, mental physical) **No**

Unprofessional Conduct (e.g. breach of confidence, record alteration, fraud, misleading advertising, arrest or conviction) **Yes**

Office Practice (e.g. failure to provide patient/medical records to patient, failure to sign death certificate, patient abandonment):

No

Other:

No

If the complaint is medical/psychological/psychotherapy treatment related, please indicate the patient has been examined/treated by another provider/professional for this same condition?:

Yes

If you responded 'Yes' to the previous question, please include provider name, address, and treatment dates.

[REDACTED]

Have you initiated or filed legal action regarding this incident?

No

What is the desired outcome of your complaint submission?

Investigation for patient abandonment

Respondent

License Type:

Physician's and Surgeon's

License Number:

171284

First Name:

MAHMOUD

Second Name:

MOHAMED ABDEL

Last Name:

ABORABEH

Address:

14541 Delano St

VAN NUYS, CA

91411-2820

US

Complainant

First Name:

Nicolas

Last Name:

Hernandez

Gender:

Male

Address:

[REDACTED]

LOS ANGELES

[REDACTED] CA

9 [REDACTED]

US

Phone Number:

[REDACTED]

E-mail Address:

[REDACTED]

Attachments

Dr._Mahmoud_Prescription_Lapse.png

Dr._Mahmoud_Gabapentin_Prescription.png

ER VISIT FOR 12 DAY LAPSE.pdf

Exhibit 8

6/12/26 7:37 AM

Page 1 of 4

Board:	California Board of Registered Nursing - BRN
License Type:	Registered Nurse - RN
Complaint Number:	4002026009197
Incident Date:	05/26/2026

Description:

I am writing to formally request a review of the professional conduct and clinical practices of Kenneth Emeka Ezenwa, NP (License #95080178) regarding a telehealth encounter on May 26, 2026. This submission focuses on Mr. Ezenwa's clinical performance and adherence to the Nursing Practice Act. Please note that a related administrative privacy complaint is currently under review by the OCR (Ticket # [REDACTED] 72).

Professional Conduct and Clinical Oversight Issues:

Failure to Review Clinical History: At the start of our appointment, Mr. Ezenwa stated he had no access to my medication history or records. He remained unprepared to proceed until I offered to read him the dates from my personal CURES report, which necessitated him placing me on hold to contact his office. This indicates a significant failure to review patient records prior to providing clinical services.

Improper Prescribing and Overriding Patient Boundaries: During the encounter, I explicitly communicated that I was in the process of a "warm hand-off" for my Diazepam taper from my current provider and clearly declined the addition of Sertraline. Mr. Ezenwa repeatedly refused to accept this boundary, persisting in his demand for the medication until I ultimately consented solely to end the circular and unproductive exchange. This behavior reflects a disregard for patient boundaries and a failure to engage in the collaborative, informed decision-making required by the Nursing Practice Act. My pharmacy records confirm this prescribing activity, which occurred despite my clear, documented objections during the appointment.

Substandard Administrative Environment: The appointment was characterized by a chaotic communication breakdown, involving multiple overlapping calls from clinic lines during the scheduled telehealth window. This prevented a professional clinical environment and impeded my ability to access care.

Request for Action:

I request that the Board investigate Mr. Ezenwa's adherence to the Nursing Practice

Act regarding record-keeping requirements, the maintenance of professional clinical standards, and the appropriate use of his prescribing (furnishing) authority.

Have you filed a complaint with any other government agency regarding this incident?		Yes
Agency Name:		Office for Civil Rights
Person Helping You:		N/A
Phone Number:		N/A
Case Number:		72
Does this complaint concern a child custody issue?		No
Was the person named in this complaint appointed by the court to prepare a custody recommendation to the court?		No
If children are involved in this case, do you have joint legal custody of the child/children involved in this case?		No
Have you tried to resolve or mediate this complaint?		No
Reason for treatment:		Continuation of care/Med management
Patient Name:		Nicolas Hernandez
Date of Birth:		(mm/dd/yyyy)
Relationship to the patient:		Self
Substandard Care (e.g. misdiagnosis, negligent treatment, delay in treatment, etc.)		Yes
Prescribing Issues (e.g. excessive/under prescribing, Internet)		Yes
Unlicensed Provider or Aiding/Abetting unlicensed practice		No
Sexual Misconduct		No
Provider (e.g. Physician, Psychologist, Psychotherapist etc.) Impairment (e.g.drug, alcohol, mental physical)		No
Unprofessional Conduct (e.g. breach of confidence, record alteration, fraud, misleading advertising, arrest or conviction)		Yes
Office Practice (e.g. failure to provide patient/medical records to patient, failure to sign death certificate, patient abandonment):		Yes

If the complaint is medical/psychological/psychotherapy treatment related, please indicate the patient has been examined/treated by another provider/professional for this same condition?:

Yes

If you responded 'Yes' to the previous question, please include provider name, address, and treatment dates.

[REDACTED]

Have you initiated or filed legal action regarding this incident?

No

What is the desired outcome of your complaint submission?

Investigate license conduct

[REDACTED]

License Type: **Registered Nurse - RN**

License Number: **95080178**

First Name: **KENNETH**

Second Name: **EMEKA**

Last Name: **EZENWA**

Address:

,

[REDACTED]

First Name: **Nicolas**

Last Name: **Hernandez**

Gender: **Male**

Address: [REDACTED]

[REDACTED] CA

9 [REDACTED]

US

Phone Number: [REDACTED]

E-mail Address: [REDACTED]

[REDACTED]

Carepoint-Pharmacy-Record.pdf **Record of Carepoint Pharmacy showing Sertraline prescription**

Ezenwa-appointment-call-logs.pdf **Call logs showing misalignment of office staff**

[REDACTED]

Exhibit 9

Name: Nicolas Hernandez | DOB: | MRN: E | PCP: Romeo Castillo, MD | Legal Name: Nicolas Hernandez

Progress Notes

Signed Jul 29, 2026

Progress Notes by Paul Johnson at 7/29/2026 11:26 AM

BARTZ-ALTADONNA COMMUNITY HEALTH CENTER
43322 Gingham Avenue
Lancaster, CA, 93535

CHIEF COMPLAINT

Nicolas presents for Complete Physical Exam (Pt presents to Establish care/ Physical./Pt presents with c/o being previously prescribed diazepam and last taken on 7/8. Pt reports he went into AVMC on 7/13, 7/14 and 7/15 due to withdrawal symptoms. Pt reports his anxiety is worse after his last dose of diazepam)

SUBJECTIVE

Nicholas Hernandez presents to the clinic today to establish care, agrees to labs with telephone follow up

Numerous ED visits re benzodiazepine withdrawal

He was working with a psychiatric provider and receiving benzodiazepines and suboxone for his anxiety and fentanyl abuse

Used fentanyl x 4-5 years

Off fentanyl since August last year

I declined to provide benzodiazepines to the patient, he has withdrawn several times, is now buying it on the street, We discussed this and I discourage this, We discussed brain receptors, what substances appeal and why.

Per him he is on several strengths of suboxone, will let addiction medicine manage this as well

Will refer to Addiction medicine for management

Follow up in two weeks by phone re lab results

Currently not working, has living space, has food.

Modality Used:

In person and in clinic

Interpreter needed: **No**

Past medical History

Past Medical History:

Diagnosis	Date
• Substance abuse (CMS & HHS-HCC)	

Social History

Social History

Tobacco Use

- Smoking status: Never
- Passive exposure: Never
- Smokeless tobacco: Never

Vaping Use

- Vaping status: Never Used

Substance Use Topics

- Alcohol use: Never
- Drug use: Never

Family History

Family History

Problem	Relation	Name	Age of Onset
• No Known Problems	Mother		
• Diabetes	Father		

Sexual Activity

Social History

Substance and Sexual Activity
Sexual Activity Not Currently

Review of Systems

Constitutional: Positive for activity change.

HENT: Negative.

Eyes: Negative.

Respiratory: Negative.

Cardiovascular: Negative.

Gastrointestinal: Negative.

Endocrine: Negative.

Genitourinary: Negative.

Musculoskeletal: Negative.

Skin: Negative.

Allergic/Immunologic: Negative.

Neurological: Negative.

Hematological: Negative.

Psychiatric/Behavioral: Positive for sleep disturbance. The patient is nervous/anxious.

OBJECTIVE

Vitals:

07/29/26 1056
BP: 102/69
BP Site: Right Arm
BP Position: Sitting
BP Cuff Regular Adult
Size:
Pulse: (!) 108
Resp: 19
Temp: 97.5 °F (36.4 °C)
TempSrc: Temporal
SpO2: 96%
Weight: 111 lb 9.6 oz (50.6 kg)
Height: 5' 6" (1.676 m)

Body mass index is 18.01 kg/m².

BP 102/69 at 7/29/2026 10:56 AM

BP 129/87 at 10/3/2025 3:36 PM

Allergies

Allergen	Reactions
• Tramadol	Other Severe
<i>Pt reports seizures occur</i>	

Last 3 HbA1c

No results found for: "HGBA1C"

Last 3 LDL

No results found for: "LDL"

Last 3 PHQ-9 scores

PHQ-9 Total Score (Auto Calculated) 8 at 7/29/2026 10:36 AM

PHQ-9 Total Score (Auto Calculated) 7 at 10/3/2025 3:27 PM

PHYSICAL EXAM

Physical Exam

Constitutional:

Appearance: Normal appearance.

Comments: **underweight**

HENT:

Head: Normocephalic and atraumatic.

Right Ear: Tympanic membrane, ear canal and external ear normal.

Left Ear: Tympanic membrane, ear canal and external ear normal.

Mouth/Throat:

Mouth: Mucous membranes are moist.
Pharynx: Oropharynx is clear.

Eyes:

Extraocular Movements: Extraocular movements intact.
Conjunctiva/sclera: Conjunctivae normal.
Pupils: Pupils are equal, round, and reactive to light.

Cardiovascular:

Rate and Rhythm: Regular rhythm. Tachycardia present.
Pulses: Normal pulses.
Heart sounds: Normal heart sounds.

Pulmonary:

Effort: Pulmonary effort is normal.
Breath sounds: Normal breath sounds.

Abdominal:

General: Bowel sounds are normal.
Palpations: Abdomen is soft.

Musculoskeletal:

General: Normal range of motion.
Cervical back: Normal range of motion and neck supple.

Skin:

General: Skin is warm and dry.
Capillary Refill: Capillary refill takes 2 to 3 seconds.

Neurological:

General: No focal deficit present.
Mental Status: He is alert and oriented to person, place, and time.

Psychiatric:

Behavior: Behavior normal.

ASSESSMENT & PLAN

Problem List

None

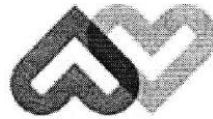
Visit Diagnoses

	Codes
Well adult exam - Primary	ICD-9-CM: V70.0 ICD-10-CM: Z00.00
Relevant Orders	
HIV 1/2 AG & AB W/RFLX (4TH GEN) Routine	
LIPIDS W RFLX TO DIRECT LDL Routine	
THYROID PANEL WITH TSH Routine	
URINALYSIS, COMPLETE W/REFLEX TO CULTURE Urine Routine	
HGA1C W/EAG Routine	
COMPREHENSIVE METABOLIC PANEL Routine	
BLOOD COUNT COMPLETE AUTO&AUTO DIFRNTL WBC Routine	
ACUTE HEPATITIS PANEL W/RFLX Routine	
Vitamin D, 25-Hydroxy Routine	
RPR (MONITOR) W/REFL TITER Routine	
History of substance abuse (CMS & HHS-HCC) (Chronic)	ICD-9-CM: 305.93 ICD-10-CM: F19.11
Relevant Orders	
REFERRAL TO ADDICTION MEDICINE	
Anxiety	ICD-9-CM: 300.00 ICD-10-CM: F41.9
Relevant Orders	
REFERRAL TO ADDICTION MEDICINE	

The following were addressed in today's visit:
Regulatory documentation not addressed.

Return in about 2 weeks (around 8/12/2026) for telephone follow up re lab results.

Exhibit 10



Antelope Valley
Medical Center

Antelope Valley Medical Center

1600 West Avenue J

Lancaster, CA 93534

661-949-5115

Discharge Instructions (Patient)

Name: HERNANDEZ, NICOLAS **Current Date:** 07/12/26 03:04:18

DOB: [REDACTED] **MRN:** [REDACTED] **80 FIN:** [REDACTED] 04

Diagnosis: 1:Anxiety; 2:Benzodiazepine dependence; 3:Opioid dependence

Visit Date: 07/12/26 01:32:08

Address: [REDACTED] CA 9[REDACTED]

Phone:

Primary Care Provider: Bartz-Altadonna, Community Health Ctr-Palmdale AVH **Phone:**
(661)874-4050

Emergency Department Providers:

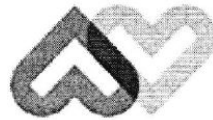
Provider Name	Role
Meredith Graham	ED APP

Antelope Valley Hospital would like to thank you for allowing us to assist you with your healthcare needs. The following instructions include patient education materials and information regarding your injury/illness.

The examination and treatment you have received today has been on an emergency basis only and has not been intended to be a substitute for complete medical care. Since it is impossible to recognize and treat all elements of injury or illness in a single visit, for your protection in preventing possible complications, if you are not clearly improving in the next 12 hours, you should be seen for a re-check at this emergency room or at your own doctor's office immediately.

IF YOU FEEL YOUR CONDITION HAS AGAIN BECOME AN EMERGENCY, RETURN TO THIS DEPARTMENT AT ONCE, DAY OR NIGHT.

IF YOU EXPERIENCE SHORTNESS OF BREATH OR CHEST PAIN. PLEASE CALL 9-1-1.



Antelope Valley Medical Center

Antelope Valley Medical Center

1600 West Avenue J

Lancaster, CA 93534

661-949-5115

Discharge Instructions (Patient)

Name: HERNANDEZ, NICOLAS **Current Date:** 07/14/26 17:25:06

DOB: [REDACTED] **MRN:** [REDACTED] 80 **FIN:** [REDACTED] 90

Diagnosis: 1:Anxiety; 2:Benzodiazepine dependence

Visit Date: 07/14/26 15:08:22

Address: [REDACTED] CA 9 [REDACTED]

Phone:

Primary Care Provider: Bartz-Altadonna, Community Health Ctr-Palmdale AVH **Phone:**
(661)874-4050

Emergency Department Providers:

Provider Name	Role
Jennifer Latey	ED APP

Antelope Valley Hospital would like to thank you for allowing us to assist you with your healthcare needs. The following instructions include patient education materials and information regarding your injury/illness.

The examination and treatment you have received today has been on an emergency basis only and has not been intended to be a substitute for complete medical care. Since it is impossible to recognize and treat all elements of injury or illness in a single visit, for your protection in preventing possible complications, if you are not clearly improving in the next 12 hours, you should be seen for a re-check at this emergency room or at your own doctor's office immediately.

IF YOU FEEL YOUR CONDITION HAS AGAIN BECOME AN EMERGENCY, RETURN TO THIS DEPARTMENT AT ONCE, DAY OR NIGHT.

IF YOU EXPERIENCE SHORTNESS OF BREATH OR CHEST PAIN. PLEASE CALL 9-1-1.

Palmdale Regional Medical Center
38000 Medical Center Drive
Palmdale, CA 93551
<http://www.palmdaleregional.com/>
(661) 382-5000

Name: HERNANDEZ, NICOLAS

Age: [REDACTED] Years **Date of Birth:** [REDACTED]

MRN: PRM [REDACTED] **FIN:** PRM [REDACTED] **52 Arrival Time:** 7/15/2026 07:31:00

Diagnosis: Benzodiazepine withdrawal

Emergency Department Care Team:

Provider: Ed staff, Physician MD

The Emergency Department physician has reviewed the information that you have provided concerning medications that have been prescribed previously and found there to be no conflict with any therapy recommended by the Emergency Department physicians. Unless instructed by the Emergency Department physician to discontinue specific medications, you should continue medications prescribed by your regular doctor and follow-up with your doctor or with the physician/facility recommended by the ED as appropriate.

If you plan on operating a motor vehicle or using any dangerous equipment within the next several hours, please check with your physician or nurse to make sure that none of the medicines that you received in the Emergency Department could interfere with your performance of these tasks.

The physicians and staff of the PRM Center encourage you to lead a healthy lifestyle. If you smoke, we strongly urge you to quit. Contact your local American Lung Association for additional information.

Allergies:

No Known Allergies

Vaccination/Immunologic Information

Prescriptions Given to Patient/Caregiver(s):

What	How Much	When	Instructions	Next Dose
OLANzapine (OLANzapine 5 mg oral tablet, disintegrating) Sent electronically: CVS/pharmacy #9785	5 mg By Mouth	Every 6 hours as needed; Other (See Comments)	Provider Note: 1 Tabs Oral q6H, PRN: Other (See Comments)	

Prescriptions sent electronically to:

CVS/pharmacy #9785 **OPEN 24 HOURS**
2006 W Avenue J
Lancaster, CA 93536-3913
(661) 945-2729

Patient Name: HERNANDEZ, NICOLAS
MRN: PRM [REDACTED] 27

NOTE: Permanent Medical Record
FIN: PRM [REDACTED] 52

07/15/2026 12:23:24



June 26, 2026

Gavin Newsom, Governor
State of California
Health and Human Services Agency
DEPARTMENT OF MANAGED HEALTH CARE
980 9th Street, Suite 500
Sacramento, CA 95814
Toll-Free: 1-888-466-2219 | Fax: 916-255-5241
Email: helpline@dmhc.ca.gov
www.DMHC.ca.gov

NICOLAS HERNANDEZ

[REDACTED]
[REDACTED] CA 9 [REDACTED]

Patient: Nicolas Hernandez
DMHC#: [REDACTED] 44
Health Plan: Local Initiative Health Authority for L.A. County (L.A. Care Health Plan)

Dear Nicolas Hernandez:

Thank you for sending your Independent Medical Review (IMR)/Complaint Form to the Department of Managed Health Care (Department).

The Department reviewed your complaint in which you expressed dissatisfaction with your experience with L.A. Care Health Plan. In your complaint, you stated that you attempted to utilize L.A. Care's mandated non-emergency medical transportation benefits to travel to the pharmacy to pick up critical, time-sensitive prescription medications. However, the health plan's transportation phone lines subjected you to excessive, unlawful hold times, effectively denying you timely access to covered prescription services. Our review included your complaint documents, health plan response, and Evidence of Coverage (health plan contract).

Quality Concerns

For the quality concerns in your complaint, L.A. Care Health Plan referred your concerns to L.A. Care Health Plan's quality assurance program for further investigation. This means L.A. Care Health Plan will evaluate the quality of your experience.

California law requires health plans to identify and correct any quality problems. While the Department monitors health plan quality assurance programs to make sure they are addressing quality concerns, the Department cannot provide the specific outcome of the quality review in relation to your complaint.

Please be advised, the Department and/or health plans are not responsible for the professional practice of individual doctors, nurses, and other providers. If you would like to pursue a complaint against a specific provider, please contact the agency listed below.

To file a complaint against a specific non-emergency medical transportation provider, you may contact the California Department of Health Care Services at:

Department of Health Care Services
Office of the Ombudsman
Telephone: 888-452-8609
MMCDOmbudsmanOffice@dhcs.ca.gov
DHCSNMT@dhcs.ca.gov

The Department determined that L.A. Care Health Plan complied with your health plan contract and California health plan law about the issue(s) in your complaint.

The Department's review of your complaint does not prevent you from pursuing other legal options that may be available, including contacting an attorney.

If you have questions, please call us toll-free at 1-888-466-2219. You may also go to our website at www.DMHC.ca.gov, which has more information about the Department and consumer health care rights in California.

Sincerely,

Standard Complaint Branch
Help Center
Department of Managed Health Care

cc: Local Initiative Health Authority for L.A. County



1 877-344-2858



Outgoing call HD

Jun 23, 12:43 PM



Outgoing call HD

Jun 23, 1:16 PM

22min 20sec



Call



Message



1 888-466-2219



Outgoing call HD
Jun 19, 11:31 AM

14min 17sec



Call



Message

Exhibit 14

What is your medical condition or doctor's diagnosis? (Please be specific, 500 character limit) *

Severe physiological benzodiazepine dependence requiring a strictly managed, medically supervised titration and clinical taper. Patient faces an immediate, documented risk of acute central nervous system destabilization, acute withdrawal syndrome, and life-threatening complications, including seizures, if unsupervised.

What medical treatment(s)/service(s) and/or medication(s) are you asking for? (Please be specific, 500 character limit) *

Immediate authorization for an expedited, directed clinical referral to a qualified provider (in-network or out-of-network) for mandatory physician oversight, monitoring, and clinical management of a high-risk medical taper. Requesting immediate network placement with a supervising physician to prevent severe neurological complications

Briefly describe the problem you are having with your health plan. For example, explain if the problem is a denied treatment, an unpaid bill, trouble getting an appointment or medication, or if your coverage has been cancelled by the health plan. (2500 character limit) *

Urgent/Expedited IMR request for immediate threat to safety. Health plan constructively denied care and failed its statutory duty to provide a network physician to manage a high-risk psychiatric taper.

Patient is undergoing a medical taper from long-term benzodiazepines due to physiological dependence. As certified by the specialist, unmonitored titration presents an immediate risk of acute CNS destabilization, severe withdrawal, and life-threatening grand mal seizures. To avoid severe danger, patient sought emergency out-of-network care out-of-pocket via [REDACTED]. This continuity care has terminated. Provider issued a final 30-day prescription on June 8, 2026, and canceled the subscription. Patient faces a total loss of medical supervision in under 30 days while mid-taper.

JURISDICTION COVER: DMHC previously closed case # [REDACTED] 55 on April 8, 2026, mislabeling this a Medi-Cal Rx pharmacy issue. This is false. Patient is NOT disputing medication coverage or claims; patient possesses the medication. This is a FORWARD-LOOKING NETWORK FAILURE. Patient demands immediate prospective authorization for a directed referral to an in/out-of-network physician for mandatory clinical monitoring before the June prescription runs out.

Patient complied with DMHC's April 8 directive and exhausted CDSS State Hearings (SHN-[REDACTED] 96 / SHN-[REDACTED] 55). CDSS denied expedited review April 14, and Conlan FFS moved to dismiss May 7, claiming the file "is still in review," bypassing the AB 3275 30-day mandate. Because the state hearing system entered indefinite delay, DMHC retains exclusive jurisdiction over this network crisis. Immediate state intervention is required to prevent severe neurological injury.

Exhibit 15

Name: Nicolas ■ Hernandez | DOB: ■■■■■■ | MRN: E ■■■■■■ | PCP: Romeo Castillo, MD | Legal Name: Nicolas ■ Hernandez

Progress Notes

Signed Oct 3, 2025

Progress Notes by Jasmin Garcia at 10/3/2025 3:38 PM

BARTZ-ALTADONNA COMMUNITY HEALTH CENTER
43322 Gingham Avenue
Lancaster, CA, 93535

CHIEF COMPLAINT

Nicolas presents for New Patient (new pt-physical/ med refill questions about med)

SUBJECTIVE

Nicholas is a ■■■ yr old male here for a physical. Mhx of substance abuse with illicit drugs (currently suboxone and clonazepam). Reports seizing without any clonazepam, on 1 mg. Has being getting med refills of suboxone through the ER. Denies any chest pain, hallucinations, or sob. No further complaints at this time.

Interpreter needed: No

Past medical History

Past Medical History:

Diagnosis	Date
• Substance abuse	

Social History

Social History

Tobacco Use

- Smoking status: Never
- Passive exposure: Never
- Smokeless tobacco: Never

Vaping Use

- Vaping status: Never Used

Substance Use Topics

- Alcohol use: Never
- Drug use: Never

Family History

Family History

Problem	Relation	Name	Age of Onset
• Diabetes	Father		

Sexual Activity

Social History

Substance and Sexual Activity

Sexual Activity Not Currently

Review of Systems

Constitutional: Negative for activity change, appetite change, chills, fatigue, fever and unexpected weight change.
HENT: Negative for congestion, ear discharge, ear pain, hearing loss and sore throat.
Eyes: Negative for discharge and visual disturbance.
Respiratory: Negative for cough, shortness of breath and wheezing.
Cardiovascular: Negative for chest pain and palpitations.
Gastrointestinal: Negative for abdominal distention, abdominal pain, blood in stool, heartburn and indigestion.
Endocrine: Negative for polydipsia, polyphagia and polyuria.
Genitourinary: Negative for difficulty urinating, dysuria, enuresis, hematuria, penile discharge and penile pain.
Musculoskeletal: Negative for arthralgias, neck pain and neck stiffness.
Skin: Negative.
Allergic/Immunologic: Negative for food allergies.

Neurological: Negative for dizziness, facial asymmetry, speech difficulty, light-headedness and headaches.
Psychiatric/Behavioral: Negative. Negative for agitation, hallucinations and suicidal ideas. The patient is not nervous/anxious.

OBJECTIVE

Vitals:

10/03/25 1536
BP: 129/87
BP Site: Right Arm
BP Position: Sitting
BP Cuff: Regular Adult
Size:
Pulse: 92
Resp: 18
Temp: 98.3 °F (36.8 °C)
TempSrc: Forehead
SpO2: 95%
Weight: 123 lb 6.4 oz (56 kg)

There is no height or weight on file to calculate BMI.

BP 129/87 at 10/3/2025 3:36 PM

No Known Allergies

Last 3 HbA1c

No results found for: "HGBA1C"

Last 3 LDL

No results found for: "LDL"

Last 3 PHQ-9 scores

PHQ-9 Total Score (Auto Calculated) 7 at 10/3/2025 3:27 PM

PHYSICAL EXAM

Physical Exam

Constitutional:

General: He is not in acute distress.
Appearance: Normal appearance. He is normal weight.

HENT:

Head: Normocephalic.
Right Ear: Tympanic membrane, ear canal and external ear normal.
Left Ear: Tympanic membrane, ear canal and external ear normal.
Nose: Nose normal. No congestion or rhinorrhea.
Mouth/Throat:
Mouth: Mucous membranes are moist.
Pharynx: Oropharynx is clear.

Eyes:

Extraocular Movements: Extraocular movements intact.
Pupils: Pupils are equal, round, and reactive to light.

Neck:

Thyroid: No thyroid mass or thyroid tenderness.

Cardiovascular:

Rate and Rhythm: Normal rate and regular rhythm.
Pulses: Normal pulses.
Heart sounds: No murmur heard.

Pulmonary:

Effort: Pulmonary effort is normal. No respiratory distress.
Breath sounds: Normal breath sounds. No wheezing.

Abdominal:

General: Abdomen is flat. Bowel sounds are normal.
Palpations: Abdomen is soft.
Tenderness: There is no right CVA tenderness or left CVA tenderness.

Genitourinary:

Prostate: Normal.

Musculoskeletal:

General: No swelling, tenderness or deformity.

Cervical back: Normal range of motion. No rigidity or tenderness.

Skin:

General: Skin is warm and dry.

Capillary Refill: Capillary refill takes less than 2 seconds.

Neurological:

General: No focal deficit present.

Mental Status: He is alert and oriented to person, place, and time.

Motor: No weakness.

Psychiatric:

Mood and Affect: Mood normal.

Behavior: Behavior normal.

ASSESSMENT & PLAN

Problem List

None

Visit Diagnoses

	Codes
Routine general medical examination at a health care facility - Primary	ICD-9-CM: V70.0 ICD-10-CM: Z00.00
Relevant Orders COMPREHENSIVE METABOLIC PANEL Routine	
Encounter for screening for diabetes mellitus	ICD-9-CM: V77.1 ICD-10-CM: Z13.1
Relevant Orders HGA1C W/EAG Routine	
Screening for deficiency anemia	ICD-9-CM: V78.1 ICD-10-CM: Z13.0
Relevant Orders BLOOD COUNT COMPLETE AUTO&AUTO DIFRNTL WBC Routine	
Screening for thyroid disorder	ICD-9-CM: V77.0 ICD-10-CM: Z13.29
Relevant Orders TSH W/RFLX FREE T4 Routine	
Encounter for HCV screening test for low risk patient	ICD-9-CM: V73.89 ICD-10-CM: Z11.59
Relevant Orders ACUTE HEPATITIS PANEL W/RFLX Routine	
Encounter for screening for human immunodeficiency virus (HIV)	ICD-9-CM: V73.89 ICD-10-CM: Z11.4
Relevant Orders HIV 1/2 AG & AB W/RFLX (4TH GEN) Routine	
Screening for hematuria or proteinuria	ICD-9-CM: V82.9 ICD-10-CM: Z13.89
Relevant Orders URINALYSIS, COMPLETE W/REFLEX TO CULTURE Urine Routine	
Uncomplicated opioid dependence (Chronic)	ICD-9-CM: 304.00 ICD-10-CM: F11.20
Screening for lipid disorders	ICD-9-CM: V77.91 ICD-10-CM: Z13.220
Relevant Orders LIPIDS W RFLX TO DIRECT LDL Routine	

The following were addressed in today's visit:

Return in about 4 weeks (around 10/31/2025) for LAb results w/MD via phone appt.

Board:	California Board of Registered Nursing - BRN
License Type:	Registered Nurse - RN
Complaint Number:	4002026005943
Incident Date:	10/03/2025

Description:

Nature of Complaint: Gross Negligence, Patient Abandonment, Incompetence, and Inappropriate Delegation of Care (Abuse of Emergency Services).

I presented to Jasmin Garcia, FNP for a physical and to establish primary care for medical management of a chemical dependency (Suboxone and Clonazepam). During this visit, FNP Garcia exhibited a severe and dangerous extreme departure from the standard of care:

Failure to Treat a Life-Threatening Condition (Gross Negligence): In her own clinical notes, FNP Garcia documented my history and explicitly wrote: "Reports seizing without any clonazepam." Despite documenting an active, life-threatening seizure risk from benzodiazepine withdrawal, she provided absolutely no medical management, no tapering protocol, and no emergency medication. She dismissively concluded the notes with "No further complaints at this time," completely ignoring the fatal risk she had just documented.

Patient Abandonment & Unsafe Delay of Care: Rather than consulting her supervising physician or initiating a safe bridge, she refused to provide care and instructed me to wait 26 days (until 10/29/2025) for a brief phone consultation with an MD, leaving a high-risk patient entirely unmanaged.

Inappropriate Delegation / Abuse of Emergency Services: FNP Garcia explicitly instructed me to continue utilizing the local hospital Emergency Room to obtain my prescription refills for Suboxone for the next month. My managed care plan (L.A. Care) formally documented this instruction in a grievance resolution letter. Instructing an established patient to abuse acute Emergency Department resources for routine addiction maintenance—because the FNP refuses to provide standard primary care—is a gross violation of clinical protocols and state healthcare guidelines.

I request the Board of Registered Nursing investigate FNP Garcia for gross negligence, failure to consult a supervising physician regarding a severe seizure risk, and her unsafe instruction to utilize emergency rooms for standard prescription maintenance.

Business Name/Facility Name:	Bartz-Altadonna Community Health Center
Address Line 1:	38660 Medical Center Dr.
Address Line 2:	Unit A200
City:	Palmdale
State:	California
Zip:	93551
Hospital:	No
Home:	No
Other:	Yes
If you responded 'Yes' to the previous question, please provide a description of the location of the incident.	Community Health Center

Have you filed a complaint with any other government agency regarding this incident?	No
Does this complaint concern a child custody issue?	No
Was the person named in this complaint appointed by the court to prepare a custody recommendation to the court?	No
If children are involved in this case, do you have joint legal custody of the child/children involved in this case?	No
Have you tried to resolve or mediate this complaint?	No
Reason for treatment:	Medication Management
Patient Name:	Nicolas Hernandez
Date of Birth:	██████████ (mm/dd/yyyy)
Relationship to the patient:	Self
Substandard Care (e.g. misdiagnosis, negligent treatment, delay in treatment, etc.)	Yes
Prescribing Issues (e.g. excessive/under prescribing, Internet)	No
Unlicensed Provider or Aiding/Abetting unlicensed practice	No
Sexual Misconduct	No
Provider (e.g. Physician, Psychologist, Psychotherapist etc.) Impairment (e.g.drug, alcohol, mental physical)	No

Unprofessional Conduct (e.g. breach of confidence, record alteration, fraud, misleading advertising, arrest or conviction)

No

Office Practice (e.g. failure to provide patient/medical records to patient, failure to sign death certificate, patient abandonment):

Yes

Other:

No

If the complaint is medical/psychological/psychotherapy treatment related, please indicate the patient has been examined/treated by another provider/professional for this same condition?:

Yes

If you responded 'Yes' to the previous question, please include provider name, address, and treatment dates.

[REDACTED]

Have you initiated or filed legal action regarding this incident?

No

License Type:

Registered Nurse - RN

First Name:

Jasmin

Last Name:

Garcia

Gender:

Female

Address:

38660 Medical Center Dr.

Unit A200

Palmdale, CA

93551

US

Phone Number:

(661) 874-4050

First Name:

Nicolas

Last Name:

Hernandez

Gender:

Male

Address:

[REDACTED]

LOS ANGELES

[REDACTED] **CA**

9 [REDACTED]

US

Phone Number:

[REDACTED]

E-mail Address:



LA CARE FAILURE.pdf

**L.A. Care Grievance - Proof of instruction to
use ER for maintenance**



Exhibit 17



L.A. Care
HEALTH PLAN

For All of L.A.

November 04, 2025

NICOLAS HERNANDEZ
[REDACTED]
[REDACTED] CA, 9[REDACTED]

Member Name: NICOLAS [REDACTED] HERNANDEZ
Member ID: [REDACTED]
Reference Number: [REDACTED] 28
Line of Business: Medi-Cal
Date of Service: October 05, 2025
Regarding: Access to care



Dear NICOLAS HERNANDEZ,

Thank you for bringing your concern(s) to our attention. L.A. Care Health Plan received your grievance on October 05, 2025. The following describes the results of our review:

Concern/s:

You told us you have tried to find an addiction specialist, but many providers either said they could not accept you as a patient or were not taking new members. You also said your Primary Care Physician (PCP) told you to go to the emergency room to keep getting your medicine, and you feel that is not the best way to get treatment.

Resolution:

We reviewed your concern and confirmed that L.A. Care Health Plan (L.A. Care) covers treatment for substance use disorders. This includes help for alcohol or drug use through outpatient care, residential programs, detox, and medicines for addiction treatment (also called MAT).

These services are offered through the Los Angeles County Department of Public Health – Substance Abuse Prevention and Control (DPH-SAPC). They can connect you with treatment centers that meet your needs. You can call them any time at 1-800-854-7771 for an assessment and referral.

L.A. Care can also help arrange medicines for addiction treatment through your PCP or, if needed, at a hospital or emergency room. If you would like help connecting to services, please call L.A. Care Member Services at 1-888-839-9909 (TTY: 711).

Thank you for bringing your concerns to us and allowing us to help.

